7.4 Renal and Ureteric Calculi

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain comes in waves from my back to my groin." • Gives the pain 10/10 **Objective** • Restless, cannot get comfortable • Blood in the urine • Nauseated	**ACTUAL** **Acute pain** related to obstruction and spasm of the ureter by a calculus as evidenced by severe colicky flank pain.	Pain 3/10 or less within 1 hour of analgesia.	• Give analgesia (NSAID or opioid) as prescribed and check again. • Unless there is a reason not to, encourage 2–3 L/day of fluid. • Strain every drop of urine and keep any stone for analysis. • Encourage walking to help the stone pass.	Pain controlled: **goal met**.
Subjective • "I pass small amounts of urine often." **Objective** • Going often • Blood in the urine	**ACTUAL** **Impaired urinary elimination** related to ureteric obstruction as evidenced by frequency and hematuria.	Passes urine normally once the stone has passed or been removed.	• Keep an I&O chart. • If he passes no urine at all, report it at once. • Get him ready for lithotripsy or ureteroscopy as ordered.	Voiding normally: **goal met**.
Subjective • "I drink very little water at work." **Objective** • Has had a stone before • Works in the heat and drinks little	**RISK** **Risk for ineffective health self-management** as evidenced by risk factors: low fluid intake and lack of knowledge of prevention.	Can explain how to stop stones coming back.	• Teach him to drink enough to pass 2–2.5 L of urine a day. • Give diet advice that fits his type of stone. • Tell him to come in urgently if he gets fever with flank pain.	Explains prevention: **goal met**.